

PUBLIC COMMENT

RE: RIN 0955–AA13, Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care (FR Doc. 2025–23641)

Submitted via: Regulations.gov

I am a senior leader responsible for digital health and inclusion-focused innovation at a safety-net academic medical center in Brooklyn. I am developing an institutional framework for responsible AI governance and adoption, informed by direct implementation experience deploying patient-facing AI tools in a safety-net setting. I submit these comments in my individual professional capacity. I am responding to RFI **Question 2** (policy priorities), **Question 9** (patient and caregiver needs), and **Question 10** (research priorities).

This comment focuses on patient- and caregiver-facing clinical AI: portal chatbots, voice agents, automated intake workflows, and AI-enabled outreach used directly by patients and caregivers. These tools can improve access and efficiency, but those benefits will not reach everyone unless tools are built for real-world conditions—limited English proficiency, low health literacy, disability access needs, and uneven digital access. In safety-net settings, the most helpful federal actions are practical requirements that protect patients without imposing unrealistic burden.

Existing civil rights and accessibility obligations apply to AI-enabled care

HHS should make clear that AI-enabled patient communication does not reduce or replace existing legal responsibilities. Title VI and Section 1557, the ADA and Section 508, and the Plain Writing Act apply to AI-enabled communication pathways just as they apply to in-person, written, and telephonic communication. HHS can accelerate adoption by translating these obligations into baseline requirements that are clear for vendors and feasible for under-resourced providers.

Four baseline requirements for patient-facing AI (RFI Q2 and Q9)

1. Transparency

Patients should be told at the start of an interaction that AI is involved, what it can and cannot do, and how to reach a person at any time. *Recommended baseline*: plain-language disclosure at the start of the interaction, plus a persistent “get help from a person” option.

2. Language access

In safety-net settings, language access is a basic safety requirement. If disclosures and instructions are only available in English, patients with limited English proficiency are shut out of safe use. *Recommended baseline*: support English and the top 2–3 languages by patient volume at the site, with an easy language switch and a clear pathway to human help in all supported languages.

3. Plain language

Transparency only works if patients understand what they are being told. Patient-facing AI should be written at or below a 6th-grade reading level, consistent with the Plain Writing Act, and should prioritize short, single-task instructions. *Recommended baseline*: plain-language scripts, one action per step, and brief comprehension supports for critical actions (for example, “Would you like me to repeat that?”).

4. Human escalation

AI tools should not become a gatekeeper to care. If a patient requests help or an issue remains unresolved after one clarification attempt, the tool should immediately route the patient to appropriate staff—without additional automated menus. *Recommended baseline*: immediate escalation on request, with appropriate routing (nurse, pharmacist, scheduling), and clear emergency guidance when clinically appropriate.

A brief example: a portal chatbot provides accurate information but only in English, with high reading complexity and no confirmation of understanding. A caregiver misunderstands next steps, leading to delayed care. This is preventable through multilingual support, plain language, comprehension prompts, and reliable escalation.

Pre-deployment readiness and ongoing monitoring (RFI Q2)

Before go-live, organizations should define readiness criteria in advance, stress-test predictable failure modes through structured scenario-based simulation (covering language, literacy, and device barriers), and verify performance against explicit Go/No-Go thresholds. Informal usability checks are not sufficient.

Once in use, providers and vendors should monitor completion, fallback, and escalation rates—disaggregated by language group—as early warning indicators of uneven performance. HHS should also encourage organizations to integrate AI-related risks into existing patient safety monitoring systems, classifying AI workflow incidents as high-impact when they could affect clinical decisions, documentation accuracy, or care access.

Recommended baseline: if fallback or escalation rates exceed 20% for any language group for two consecutive months, require corrective action plans and pause high-risk pathways until issues are resolved.

Research and development priorities (RFI Q10)

Federal grants and contracts should prioritize tools developed with and validated in diverse populations—Medicaid beneficiaries, people with disabilities, limited English proficiency populations, and safety-net settings. Too often, tools are built and validated in well-resourced systems and then deployed elsewhere with weaker performance and new access barriers.

HHS should also support scenario-based simulated caregiver persona testing as an accepted preliminary validation step in proposals for patient-facing AI tools. This pre-deployment method identifies language, literacy, accessibility, and device-specific failures before patients are exposed—reducing compliance risk, lowering remediation costs, and preventing avoidable care delays. Alongside this, HHS should fund

implementation research on adoption in resource-constrained settings and support shared infrastructure that under-resourced providers can use without prohibitive licensing costs.

Closing

I urge HHS to establish four baseline requirements— transparency, language access, plain language, and reliable escalation—via guidance regulation, or contract language. Pairing these baselines with practical pre-deployment readiness criteria, simple ongoing monitoring, and targeted research investment will protect patients, reduce avoidable burden, and improve access and outcomes across diverse communities.

Respectfully submitted,

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Submitted in individual professional capacity